OFFICIAL NEXTGEN NURSES EDITION



AHPRA RN OSCE – 10-Station Simplified Reviewer and Scripts

Created by **NextGen Nurses** | Interactive Clinical Scenarios, Elaborated Procedures & Verbatim Examiner Scripts

Universal OSCE Approach – Safe Start and Station 1 Overview

Use this basic sequence at almost every station. The universal approach gives you a predictable structure to start safely, communicate clearly, and finish with appropriate documentation and escalation. Keep your actions visible to the examiner and your language patient-centred. The following framework preserves the original content while adding cohesive transitions so you can move confidently from setup to assessment to closure within any OSCE scenario.

Before touching the patient

- [ ] **S – Scan:** Read the scenario carefully. Identify the task. Look for immediate danger or deterioration. Check allergies and relevant documentation.
- [ ] **A – Approach:** Perform hand hygiene (WHO 5 Moments). Introduce yourself and your role. Maintain privacy and dignity (close curtains/door).
- [ ] **F – Full identification:** Use at least three approved patient identifiers, according to local policy. Example: full name, date of birth and UR/MRN verified against the ID band.
- [ ] **E – Explain:** Explain what you need to do in plain language. Obtain consent. Ask about immediate concerns, pain or discomfort.

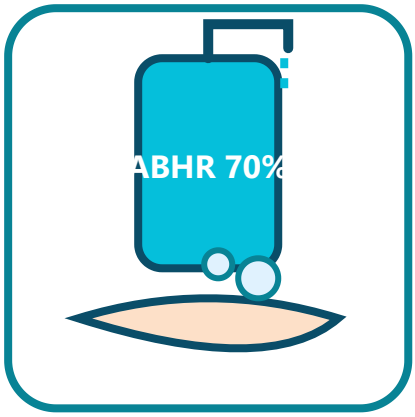
During the station

Work systematically. Maintain infection-control principles. Communicate with the patient. Recognise abnormal findings. **Do not simply collect observations — interpret them.** Link findings to likely implications and next steps, verbalizing your reasoning for the examiner.

At the end

Reassess the patient. Ensure safety and comfort (bed low, brakes locked, call bell in reach). Escalate abnormal findings appropriately. Document. Provide an ISBAR handover when required. Close the loop by confirming understanding and outlining the plan, including when you will return to reassess.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Moment 1: Hand Hygiene Before Touching a Patient

Perform visible alcohol-based hand rub (20–30 seconds) immediately upon entering the station and before approaching the patient bedside.

Station 1 — Vital Signs and Pain Assessment

Goal

Accurately assess vital signs, recognise deterioration and respond appropriately. Your performance should demonstrate systematic observation, correct technique, clinical interpretation, and timely escalation when indicated.

Assess: Core Observations

1. Respiratory rate (Normal Adult: 12–20 breaths/min)
Count respirations preferably without drawing unnecessary attention to the measurement (keep your fingers positioned on the radial pulse while observing the chest for 60 seconds).

Assess:

- [ ] Rate (breaths per minute) & Rhythm (regular vs irregular)
- [ ] Depth (normal, shallow, or deep) & Work of breathing (WOB)
- [ ] Accessory muscle use (sternocleidomastoid/intercostal)
- [ ] Ability to speak in full sentences & Skin/lip colour (cyanosis/pallor)

Clinical Pearl: A respiratory rate that is abnormal for the clinical context is often the earliest sensitive sign of acute physiological deterioration.

Station 1 — Vital Signs (Continued)

2. Oxygen saturation (SpO_2)
Check: SpO_2, Oxygen therapy if applicable, Device (nasal prongs, Hudson mask, Venturi), and Flow rate (L/min).
NextGen Nurses Elaboration: Ensure warm extremities and a reliable pulse waveform. Do not interpret SpO_2 without considering the patient's clinical condition and prescribed target range (92–96% standard; 88–92% for COPD hypercapnic retainers).

3. Heart rate (Normal Adult: 60–100 bpm)
Assess manually via radial palpation: **Rate, Rhythm (regular/irregular), and Strength (strong, bounding, weak/thready)**. If irregular, count for a full 60 seconds, assess apical pulse, and request a 12-lead ECG per local procedure.

4. Blood pressure (BP)
Ensure: Correct cuff size (bladder width 40% of arm circumference), Patient appropriately positioned (seated or supine, legs uncrossed), Arm supported at heart level, and Results interpreted against previous baseline readings and clinical condition.

5. Temperature (36.0–37.5°C)
Identify: Fever (>38.0°C), Hypothermia (<35.5°C), and Trends over time.

Station 1 — Consciousness, Pain & Escalation

6. Consciousness: Use an appropriate neurological tool such as **AVPU** (Alert, Voice, Pain, Unresponsive), or a full **GCS** when indicated.
7. Pain: Ask: "*Are you experiencing any pain at the moment?*" If yes, perform a structured PQRST pain assessment.

WHAT YOU SHOULD SAY

"Your blood pressure is lower than expected and your heart rate is elevated. I would reassess you, review the other observations and escalate this deterioration according to the facility's clinical escalation procedure."

Remember: OBSERVE → INTERPRET → ESCALATE → REASSESS. Do not collect abnormal observations and then simply leave the patient.

Common OSCE mistakes

- [ ] Forgetting hand hygiene
- [ ] Incorrect patient identification
- [ ] Not counting respiratory rate
- [ ] Recording observations without interpreting them
- [ ] Ignoring abnormal observations & failing to escalate deterioration
- [ ] Not documenting findings

Station 2 — Basic Life Support / DRSABCD

Scenario and Primary Survey

You find a patient collapsed and apparently unresponsive. Apply DRSABCD systematically, verbalising each step for safety and clarity. Keep your scene assessment dynamic—conditions can change rapidly.

D — Danger
Check for danger to: **Yourself, Patient, Others** (water on floor, electrical cords, sharps, bed position). Don gloves/PPE. Do not proceed until the area is safe or appropriately controlled.

R — Response
Assess responsiveness using verbal and tactile stimulation (COWS): *"Can you hear me? Open your eyes. What is your name? Squeeze my hands."* Apply firm shoulder tap/squeeze. If no response, escalate promptly.

S — Send for help
Press the emergency buzzer and call for emergency assistance (Code Blue / 2222). Request: **Emergency response team, AED/defibrillator, and Resuscitation trolley**. Delegate tasks clearly if help is available.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 2: DRSABCD & Early Defibrillation

Attach the AED as soon as it arrives. Minimise interruptions to chest compressions and ensure all staff stand clear during rhythm analysis and shock delivery.

Station 2 — Basic Life Support (A – B – C)

A — Airway

Flatten the bed and remove the pillow. Open and assess the airway—look inside the mouth for visible airway obstruction (secretions, vomit, loose dentures). If appropriate, use **Head tilt / chin lift**. Consider the clinical context if trauma is suspected, and use a **Jaw thrust** if cervical spine injury is a concern.

B — Breathing

Assess for normal breathing for up to 10 seconds: **Look** for chest rise, **Listen** for breath sounds, and **Feel** for expired air. If the patient is unresponsive and not breathing normally, commence CPR immediately.

C — CPR

- **Ratio (Adult):** 30 compressions : 2 breaths (heel of hand on centre of chest / lower half of sternum).
- **Compression rate:** 100–120/minute.
- **Compression depth:** Approximately 5–6 cm in an adult.
- Allow full chest recoil after every compression, minimise interruptions, and use a bag-valve-mask (BVM) or barrier ventilation device per local policy.

Station 2 — Defibrillation & Common Mistakes

D — Defibrillation
Attach AED/defibrillator pads in the anterior-lateral position as soon as available. Follow voice prompts. Ensure nobody is touching the patient during rhythm analysis or shock delivery. Command loudly: **"Stand clear!"** and perform a 360° visual check. Resume CPR (30:2) immediately after shock delivery.

Important OSCE point: Agonal gasping is NOT normal breathing. Treat as cardiac arrest if the patient is unresponsive and not breathing normally.

Common mistakes

- [ ] Delaying CPR or forgetting to call for help
- [ ] Incorrect compression hand position
- [ ] Slow (<100/min) or shallow (<5 cm) compressions
- [ ] Excessive interruptions to compressions
- [ ] Treating abnormal agonal gasping as normal breathing
- [ ] Unsafe AED use (not clearing the patient/oxygen)

Station 3 — Blood Transfusion and Reaction Management

Before transfusion: Safety Checks and Education

Check with a second qualified clinician at the bedside: **Valid order/prescription, Signed patient consent, Patent IV access (flushed with 0.9% NaCl), Baseline observations, Relevant blood results, Compatibility documentation, Blood product type, Expiry date/time, Visual integrity of product (no clots/leaks/haemolysis), 3 Patient identifiers, and Independent double-checking process** per policy. Explain the procedure in plain language and confirm understanding.

> **PATIENT EDUCATION SCRIPT**
> *"I will be monitoring you closely during the transfusion. Please tell me immediately if you develop chills, fever, difficulty breathing, itching, pain, nausea or suddenly feel unwell."*

Monitor for reaction: Recognise Early Signs

Possible features include: **Fever, Chills/rigors, Rash/urticaria, Dyspnoea, Wheeze, Hypotension, Tachycardia, Chest or back pain, Nausea, Anxiety, Haemoglobinuria, or Sudden deterioration.** Stop the transfusion immediately if a reaction is suspected and assess using ABC.

> **If you suspect a transfusion reaction: STOP – ASSESS – ESCALATE**
> **1. STOP the transfusion:** Do not continue the blood product. Clamp the roller clamp per local policy and keep the giving set intact for investigation steps.

Station 3 — Transfusion Reaction Management (Steps 2–4)

2. Assess the patient
Perform an immediate **ABCDE assessment, full repeat Vital Signs, and Focused clinical assessment**. Prioritise life-threatening airway, breathing, or circulation issues first.

3. Maintain IV access
Follow local transfusion-reaction protocol regarding IV access and compatible fluids (attach a brand-new sterile giving set primed with **0.9% Sodium Chloride** directly to the cannula hub). **CRITICAL SAFETY RULE: Do NOT flush the blood product remaining in the old tubing through to the patient!**

4. Escalate
Notify the appropriate: **Medical team, Senior nurse/Team leader, and Blood bank/transfusion service**. Activate emergency response (MET / Code Blue) if the patient is significantly deteriorating.

Station 3 — Investigation Protocol & Common Mistakes

5. Follow investigation protocol
This involves: collecting post-transfusion **Blood samples** (from opposite arm), collecting first voided **Urine sample** (for haemoglobinuria), **Returning the clamped blood bag and giving set** to pathology, and completing **Documentation and transfusion reaction forms** to support traceability and patient safety.

WHAT TO SAY

"I suspect an acute transfusion reaction. I would stop the transfusion immediately, assess ABCs and observations, maintain IV access according to protocol and urgently notify the appropriate medical and transfusion teams."

Common mistakes

- [ ] Continuing the transfusion despite symptoms
- [ ] Flushing remaining blood in the line into the patient
- [ ] Poor bedside identification checks
- [ ] Not obtaining baseline observations
- [ ] Failing to recognise early symptoms or failing to escalate

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 3: Safe Blood Product Administration

Always verify ABO/Rh compatibility and unit numbers with a second clinician at the bedside, and clamp immediately at the first sign of rigor, fever, or dyspnoea.

Station 4 — Medication Administration

Goal and Pre-administration Safety

Administer medication safely while demonstrating clinical reasoning. Before administration, check the medication order and apply your organisation's medication rights/checks. Core principles include checking the: **Right patient, Right medication, Right dose, Right route, Right time.** Also consider: **Right indication/reason, Right documentation, Right response, Patient's right to refuse, and Relevant assessment and education.**

> **CHECK ALLERGIES**
> *"Do you have any allergies or previous reactions to medications?"*

Verify against documentation and the patient's allergy wristband. If uncertain or information conflicts, pause and clarify before proceeding.

Assess before giving (Do not administer automatically):

- [ ] **Antihypertensive** → Check BP, HR, and relevant clinical parameters.
- [ ] **Insulin** → Check capillary BGL and meal timing/availability.
- [ ] **Opioid** → Assess pain score, respiratory rate, sedation level, and observations.
- [ ] **Digoxin** → Assess apical/radial pulse (>60 bpm) and electrolytes per policy.

Station 4 — Three Checks, Education & Safety

Three checks concept
Check the medication label against the chart 3 times: (1) When removing from storage, (2) During preparation, and (3) At the bedside before opening. Use independent double-checks where policy requires.

PATIENT EDUCATION (NAME, PURPOSE, PRECAUTIONS)

"This medication is paracetamol. It has been prescribed to help reduce your pain."

After administration and If something looks wrong

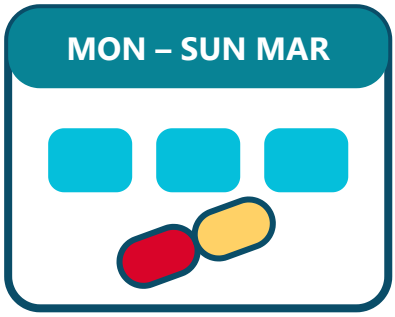
After administration: Document immediately on the MAR, monitor response, and assess for adverse effects. **If something looks wrong: DO NOT ADMINISTER. Pause and clarify.**

WITHHOLDING SCRIPT

"I would withhold administration until I have clarified the order because I cannot safely administer a medication when the prescription or clinical parameters are unclear."

Common mistakes: No allergy check • Failure to identify patient • Administering despite abnormal observations • Incorrect calculation • Poor medication knowledge • Not explaining medication • Not documenting or reassessing.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 4: The 10 Rights & 3 Bedside Checks

Never leave medications unattended at the bedside. Verify patient identity, allergies, and targeted clinical parameters before every administration.

Station 5 — IV Fluid Administration

Before commencing: Checks and Site Assessment

Check: **Prescription, Correct patient, Correct fluid, Correct volume, Correct rate, Indication, IV access patency, Allergies where relevant, Fluid balance chart, and Clinical condition.**

Inspect IV site (VIP Score) for:

- [ ] Pain or tenderness on palpation/flushing
- [ ] Redness (erythema), Warmth, or Swelling
- [ ] Leakage, Phlebitis, or Infiltration/Extravasation

Document baseline observations and flush cannula with 0.9% Sodium Chloride via ANTT before starting the infusion.

Calculate rate: Pump calculation
mL/hr = Total volume ÷ Number of hours
Example: 1000 mL over 8 hours → 1000 ÷ 8 = 125 mL/hr.
Confirm your calculation verbally for the examiner and set the pump safely, using independent verification where policy requires.

Station 5 — Patient Monitoring & Fluid Overload

Assess patient during fluids

Before and during IV fluids consider: **BP, HR, Respiratory status, SpO_2, Urine output, Fluid balance, Peripheral oedema, Lung sounds** where clinically indicated, and relevant pathology results (electrolytes/renal function). Adjust rate only with an appropriate order or standing protocol.

Watch for fluid overload (Acute Pulmonary Oedema)

Possible signs: **Increasing dyspnoea, Falling oxygen saturation, Moist lung crackles, Peripheral/sacral oedema, Rapid weight gain, and Increasing BP** in some patients. If deterioration occurs, pause/slow infusion, sit patient upright, follow emergency procedures, and escalate promptly.

WHAT TO SAY

"I would reassess the patient immediately, review the fluid therapy and urgently escalate the change in condition."

Common mistakes: Wrong infusion rate • Failing to inspect IV site • Ignoring fluid balance • Not considering cardiac/renal status • Failing to recognise fluid overload • Not documenting intake.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 5: Volumetric IV Infusion Pump Verification

Program mL/hr and Volume To Be Infused (VTBI) with a second RN, ensure roller clamps are open, and monitor hourly fluid balance.

Station 6 — ISBAR Clinical Handover

ISBAR keeps communication structured. Prepare your key facts before calling, then deliver them concisely with a clear recommendation. Aim to demonstrate situational awareness, clinical reasoning, and urgency when required.

I — Identification
Identify: Yourself, Your role/location, and the Patient (Name, Age, Bed, UR/MRN).

> *"This is Jane, RN on Ward 2. I'm calling about Mr John Smith, a 68-year-old patient in bed 4."*

S — Situation
State clearly why you are calling and the primary acute change.

> *"I'm concerned because he has developed increasing shortness of breath and his oxygen saturation has fallen."*

Station 6 — ISBAR Handover (B – A – R)

B — Background
Give only relevant background: Diagnosis, Admission reason, Surgery/procedure, Relevant history, Medications, and Allergies.

A — Assessment
Include relevant focused assessment findings and state key trends to show interpretation, not just collection of data:

> *"Respiratory rate is 30, SpO_2 is 89%, heart rate 112 and BP 95/60. He is alert but appears distressed."*

R — Recommendation

> *"I need an urgent medical review. I will continue monitoring him and escalate to the rapid response team if he deteriorates further."*

OSCE tip & Common mistakes: Don't simply dump information. Show the assessor: *What is happening? Why are you concerned? What needs to happen next?* Avoid long irrelevant history, missing current observations, or failing to communicate urgency.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 6: Structured ISBAR Clinical Communication

State your urgent clinical request clearly and close the loop by confirming the medical officer's expected arrival time and interim plan.

Station 7 — Pain Assessment and Management

Do not ask only: *"What's your pain from 0–10?"* Perform a structured assessment to characterise the pain, identify red flags, treat appropriately, and reassess for response. Use the **PQRST framework** to standardise your questioning and ensure important details are not missed.

P — Provocation / Palliation
Student prompt: *"What makes the pain worse or better?"*

Q — Quality
Ask: *"How would you describe the pain?"* Examples: Sharp, Burning, Crushing, Aching, Stabbing. Use patient language; offer descriptors only if needed.

R — Region / Radiation
Ask: *"Where is the pain?"* and *"Does it move anywhere else?"* Map radiation and consider referred pain patterns (e.g., jaw/left arm in cardiac pain; shoulder tip in diaphragmatic irritation).

Station 7 — Pain Assessment (Severity & Timing)

S — Severity

> *"On a scale of 0 to 10, where 0 is no pain and 10 is the worst pain imaginable, how would you rate it?"*

T — Timing

Ask:

- [ ] *"When did the pain start?"*
- [ ] *"Did it come on suddenly or gradually?"*
- [ ] *"Is the pain constant or intermittent (comes and goes)?"*
- [ ] *"Has it changed since it started?"*

Document onset, pattern, and duration to help differentiate acute underlying causes.

Station 7 — Additional Pain Assessment Domains

Alongside the PQRST questions, always assess the broader clinical context of the patient's pain:

- [ ] **Associated symptoms:** Nausea, vomiting, diaphoresis, shortness of breath, dizziness, bowel/urinary changes.
- [ ] **Previous pain:** Has the patient experienced this type of pain before, and what usually relieves it?
- [ ] **Effect on movement / sleep / function:** Can the patient take a deep breath, cough effectively, mobilise, or rest?
- [ ] **Vital signs:** Obtain full observations (BP, HR, RR, SpO_2, Temp, Sedation Score) before and after analgesia.
- [ ] **Relevant history:** Recent surgery, trauma, cardiovascular history, peptic ulcer disease, renal/hepatic function, and allergies.

Station 7 — Intervention, Reassessment & Red Flags

Intervention & Mandatory Reassessment

• Provide prescribed **Analgesia** plus non-pharmacological strategies: **Positioning, Rest, Heat/cold packs when appropriate, Splinting, Reassurance**.

• *Frequently forgotten:* After intervention (30–60 mins post-oral / 15–30 mins post-IV), reassess using the exact same 0–10 pain scale and document clearly.

> *"I would reassess the patient's pain and evaluate whether the intervention has been effective."*

Red Flags (Escalate Urgently!) & Common Mistakes

• **Red Flags:** Central chest pain • Sudden severe headache • Acute rigid abdominal pain • Neurological changes • Severe postoperative pain disproportionate to expected findings.

• **Mistakes:** Only obtaining a pain score • No associated symptom check • Giving analgesia without observations • No reassessment • Failure to escalate red flags.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 7: Comprehensive PQRST & Analgesia Reassessment

Combine pharmacological analgesia with positioning and comfort cares, and never close the station without verbalising your reassessment plan.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 8: Systematic A–G Patient Assessment

Approach every acutely unwell patient with the structured A–G framework to detect life-threatening airway, breathing, or circulatory compromise early.

Station 8 — A–G Patient Assessment Framework

The **NextGen Nurses AHPRA RN OSCE – 10-Station Simplified Guide** is designed to help you approach clinical scenarios using a clear and systematic **A–G Patient Assessment**. The guide breaks assessment down into simple steps so you can practise recognising abnormalities, prioritising patient safety, escalating deterioration and communicating your findings clearly.

A – Airway
Is the airway patent? Check whether the patient can speak or respond. Look and listen for signs of airway obstruction, including secretions, vomit, blood, swelling, snoring, gurgling or stridor. Consider cervical spine protection when trauma is suspected.

B – Breathing
Is breathing adequate? Assess the patient's respiratory rate, depth and pattern, oxygen saturation, work of breathing, chest expansion and skin colour. Look for signs of respiratory distress such as accessory-muscle use or cyanosis and auscultate breath sounds side-to-side when appropriate.

C – Circulation
Is circulation adequate? Assess heart rate, pulse quality and rhythm, blood pressure, capillary refill (<2s), skin colour and temperature, and other signs of poor peripheral perfusion. Look for and manage obvious bleeding. Consider urine output (≥0.5 mL/kg/hr) as an indicator of perfusion.

Station 8 — A–G Patient Assessment (D – E – F – G)

D – Disability
Assess neurological status using **AVPU** or **GCS**. Check the pupils (size, equality, reactivity to light – PEARL) and assess for new confusion, facial droop, limb weakness, seizures or other neurological changes.

E – Exposure
Expose and examine appropriately. Perform a systematic head-to-toe examination for rashes, wounds, bleeding, swelling (calf DVT check), injuries or other abnormalities. Check temperature while maintaining privacy, dignity and warmth and preventing unnecessary heat loss.

F – Fluids
Assess hydration and fluid balance. Review the patient's 24-hour fluid intake and output, urine output, IV fluids and ongoing fluid losses, including vomiting, diarrhoea, bleeding or drain output. Assess for signs of dehydration or fluid overload.

G – Glucose
Check capillary blood glucose (BGL) when clinically indicated, particularly when the patient has altered consciousness or acute deterioration. Assess for hypoglycaemia (<4.0 mmol/L) or hyperglycaemia and respond according to clinical protocol.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 8: Don't Forget 'F' (Fluids) & 'G' (Glucose)!

Always check capillary blood glucose (BGL) in any patient with new confusion, drowsiness, agitation, or acute clinical deterioration.

Station 9 — Risk Assessment

This station may involve multiple risks. Common areas include falls, pressure injury, delirium, and suicide or self-harm risk. Your approach should demonstrate targeted assessment, immediate risk reduction, clear communication, documentation, and planned review.

Falls Risk Assessment & Prevention

• **Assess:** Previous falls, Mobility, Gait, Walking aids, Culprit medications (sedatives, antihypertensives, opioids), Postural dizziness, Cognition, Vision/glasses, Toileting urgency, Environmental hazards.

• **Interventions:** Call bell within reach, Appropriate non-slip footwear, Clear clutter-free environment, Low bed with locked brakes, Mobility assistance/supervision, Falls-prevention care plan.

Pressure Injury Risk

• **Assess:** Use approved facility tool (Waterlow/Braden). Consider Mobility, Skin condition over bony prominences, Moisture/incontinence, Nutrition, Perfusion, Pressure areas.

• **Prevention:** Regular 2-hourly repositioning, Pressure-relieving air mattress/cushions, Barrier skin care, Continence management, Nutrition/hydration support.

Station 9 — Delirium & Suicide/Self-Harm Risk

Delirium

• **Look for:** Acute onset, Fluctuating cognition, Inattention, Altered consciousness, Disorientation, Behavioural changes.

• **Remember:** Delirium is acute and requires urgent investigation of an underlying organic cause: **Infection, Medication effects, Hypoxia, Dehydration, Pain, Constipation/urinary retention, or Metabolic disturbance (PINCH ME)**.

Suicide / Self-harm risk

Where relevant to the scenario, communicate calmly, empathetically, and directly. Assess immediate safety according to local policy, remove environmental hazards, and escalate concerns urgently to the appropriate clinical/mental health team. **Do not leave a patient at immediate risk without appropriate 1:1 support.**

General OSCE principle: Identifying a risk is not enough. You must demonstrate: **ASSESS → REDUCE RISK → COMMUNICATE → DOCUMENT → REVIEW.**

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 9: Proactive Environmental Risk Reduction

Lower the bed, lock the brakes, clear floor hazards, place the call bell and walking frame within reach, and ensure non-slip footwear is worn.

Station 10 — Wound Dressing and ANTT

Goal and Preparation

Perform wound care while preventing contamination using Aseptic Non-Touch Technique (ANTT).

Before procedure checklist:

- [ ] Check the wound-care order / management plan & review allergies (dressings, tapes, antiseptics).
- [ ] Explain procedure and obtain informed consent.
- [ ] Assess baseline pain (0–10) and administer prescribed analgesia 20–30 mins prior if required.
- [ ] Decontaminate dressing trolley, gather equipment, perform hand hygiene, position patient comfortably, and maintain privacy.

Remove old dressing and Observe

Put on clean non-sterile gloves and appropriate PPE. Gently remove the old dressing and observe: **Amount of exudate, Colour, Odour, Bleeding, and Condition of old dressing**. Dispose immediately into the clinical waste bag, remove gloves, and perform hand hygiene before touching the sterile field.

Station 10 — WOUND Assessment, ANTT & Escalation

Assess wound — WOUND mnemonic

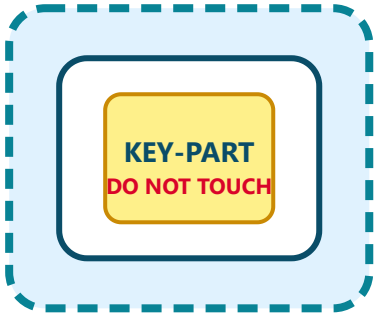
• **W — Wound bed:** Granulation (pink/healthy), Slough (yellow), Necrotic tissue (black).
• **O — Odour:** Assess for malodour after cleaning.
• **U — Underlying/surrounding skin:** Redness (erythema), Maceration, Warmth, Swelling.
• **N — Nature of exudate:** Serous, Haemoserous, Sanguineous, or Purulent.
• **D — Dimensions:** Length × Width × Depth (cm) + wound edges, pain, signs of infection, and healing progress.

ANTT, Cleaning & Applying Dressing

Protect **key parts** (sterile dressing contact surface, sterile forceps tips, sterile gauze) and the **key site** (the wound). Clean using single strokes (one swab per wipe) without contaminating cleaned areas. Apply prescribed dressing securely, dispose of waste, perform hand hygiene, ensure comfort, and document.

Signs requiring escalation: Increasing redness • Heat • Swelling • Increasing pain • Purulent discharge • Wound deterioration • Fever/systemic symptoms • Dehiscence • Unexpected bleeding.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 10: Aseptic Non-Touch Technique (ANTT)

Protect Key-Parts and Key-Sites at all times. Never touch the inner contact pad of a sterile dressing or reuse a swab across a cleaned wound bed.

Rapid OSCE Memory Page and Recognising Deterioration

Before EVERY station (H-I-I-A-E-C-P):
H – Hand hygiene • **I** – Introduce • **I** – Identify patient (3 IDs) • **A** – Allergies • **E** – Explain • **C** – Consent • **P** – Privacy.
Then: ASSESS → ACT → REASSESS → ESCALATE → DOCUMENT.

Recognising the Deteriorating Patient — ABCDE

- **A — Airway:** Is the airway clear and patent?
- **B — Breathing:** Assess RR, SpO_2, Work of breathing, Breath sounds.
- **C — Circulation:** Assess HR, BP, Perfusion/Capillary refill, Bleeding, Fluid status.
- **D — Disability:** Assess Consciousness (GCS/AVPU), Pupils when indicated, BGL when indicated.

Recognising Deterioration & Useful OSCE Phrases

E — Exposure / Everything else
Assess: Temperature, Skin, Wounds, Abdomen, Drains, and other relevant findings.
Then: CALL FOR HELP EARLY. Do not wait until you have completed every assessment if the patient is critically deteriorating.

Useful OSCE Phrases to Memorise

WHEN YOU IDENTIFY DETERIORATION

"I am concerned about this patient's clinical condition. I would remain with the patient, perform an ABCDE assessment, repeat observations and escalate according to the clinical deterioration pathway."

WHEN UNSURE ABOUT MEDICATION

"I would not administer the medication until I have clarified the order and confirmed that it is safe."

Useful OSCE Phrases — Patient Refusal & Self-Correction

WHEN THE PATIENT REFUSES

"I would explore the patient's reason for refusing, provide appropriate information, respect their decision where they have capacity, and escalate and document as required."

WHEN YOU MAKE AN ERROR DURING THE STATION (DON'T PANIC! SAY:)

"I recognise that this would compromise patient safety. I would stop, correct the issue and proceed safely."

NextGen Nurses Examiner Tip: Examiners award marks when you demonstrate safe self-awareness! If you drop a sterile forceps or forget to check an ID band before picking up a chart, verbalise the correction immediately and perform the safe action.

When You Don't Know Something & Top 12 Mark Losers

WHEN YOU DON'T KNOW SOMETHING

"I would refer to the relevant clinical guideline or medication reference and seek assistance from an appropriate senior clinician before proceeding."

Common Reasons for Losing OSCE Marks (Audit Yourself!)

- [ ] 1. Poor patient identification
- [ ] 2. Forgetting allergies
- [ ] 3. Poor hand hygiene
- [ ] 4. Breaking aseptic technique (ANTT)
- [ ] 5. Unsafe medication administration
- [ ] 6. Failure to recognise deterioration
- [ ] 7. Failure to escalate
- [ ] 8. Poor communication
- [ ] 9. Failure to reassess after intervention
- [ ] 10. Failure to document
- [ ] 11. Performing tasks mechanically without clinical reasoning
- [ ] 12. Forgetting patient dignity, privacy or consent

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Part 2: Interactive 10-Station Verbatim Scripts

Practice speaking every script aloud while filling in the interactive clinical fields on pages 44 to 66.

OSCE 10 Stations with Scripts — Station 1: Vital Signs

Student Opening Script

> *"Hello, my name is* **Your Name** *, and I am your student nurse/RN today."*

> *"Can I confirm your full name and date of birth?"*
> *"Thank you,* **Patient Name** *."*

> *"I have been asked to perform a set of observations to assess how you are doing. This will include your breathing, oxygen saturation, pulse, blood pressure, temperature and pain. Is that okay?"*

> *"Are you experiencing any pain or discomfort at the moment?"*

Station 1 Script — General Appearance & Respiratory Rate

During assessment (Verbalise to Examiner)

> *"I am initially observing the patient's general appearance, level of consciousness, skin colour and work of breathing."*

Respiratory rate

> *"I am assessing the respiratory rate, rhythm, depth and work of breathing."*

Interactive Recording:

Respiratory rate: **RR** breaths/min.

Verbalise: *"The respiratory rate is* **RR** *breaths per minute, which is* ☐ *within /* ☐ *outside the expected range."*

Station 1 Script — SpO_2, Pulse, BP, Temperature & Pain

Oxygen saturation: SpO_2: _____ % on ☐ Room air / ☐ Oxygen: _____ L/min via _____.

"The oxygen saturation is ___% on room air / ___ L/min oxygen via _____."

Pulse: _____ bpm (☐ Regular / ☐ Irregular | ☐ Strong / ☐ Weak).

"The pulse is ___ beats per minute, and it is regular/irregular and strong/weak."

Blood pressure: BP: _____ / _____ mmHg. *"The blood pressure is ___/___ mmHg."*

Temperature: _____ °C. *"The temperature is ___ degrees Celsius."*

PAIN ASSESSMENT — STUDENT ASKS:

"Are you experiencing any pain at the moment?"

•

IF NO:

"Thank you. Your current pain score is zero."

•

IF YES:

"I would like to ask you a few more questions about your pain so I can understand it better. On a scale from 0 to 10, where 0 means no pain and 10 is the worst pain you can imagine, how would you rate your pain right now?" Pain score: **/10** */10.*

Station 1 Script — PQRST Pain Assessment (P – Q – R)

P — Provocation / Palliation

- *"What were you doing when the pain started?"*
- *"Is there anything that makes the pain worse?"*
- *"Is there anything that makes it better?"*

Findings: _____

Q — Quality

- *"Can you describe what the pain feels like? For example, is it sharp, dull, aching, burning, stabbing, crushing or throbbing?"*

Quality: _____

R — Region / Radiation

- *"Can you point to where you feel the pain?"*
- *"Does the pain stay there, or does it move or spread anywhere else?"*

Location: _____ | Radiation: _____

Station 1 Script — PQRST (Severity, Timing & Symptoms)

S — Severity & T — Timing

• *"You rated your pain as _____ /10. Is that the worst it has been, or has the severity changed?"*

Current: _____ /10 | Worst: _____ /10.

• *"When did the pain start? Did it start suddenly or gradually? Is the pain constant, or does it come and go? How long does it usually last?"*

Onset: _____ | Duration/pattern: _____

Associated symptoms and Interpretation

• *"Are you experiencing any other symptoms with the pain?"*

Associated symptoms: _____

• **For chest pain:** Ask about shortness of breath, sweating (diaphoresis), nausea/vomiting, dizziness, palpitations, radiation to the arm, shoulder, back, neck or jaw.

• **For abdominal pain:** Ask about nausea/vomiting, bowel changes, urinary symptoms, appetite, abdominal distension.

Station 1 Script — Clinical Interpretation and Action

ROUTINE PAIN MANAGEMENT SCRIPT

"I would review the patient's prescribed pain-management plan and provide appropriate pharmacological or non-pharmacological interventions within my scope of practice."

If pain is new, severe, unexpected or associated with deterioration:

"I am concerned about the patient's pain because it is **severe / acute / associated with abnormal obs** *."*

"I would remain with the patient, perform an ABCDE assessment, repeat the relevant observations and escalate promptly to the appropriate clinician."

Station 1 Script — Acute Red Flag Example & Station Closing

Important Example: Acute Cardiac Chest Pain

If the patient reports **central crushing chest pain 8/10 radiating to the left arm with shortness of breath and diaphoresis** — this requires urgent escalation rather than completing routine observations:

> *"I am concerned about this patient's acute chest pain and associated symptoms. I would stop the routine assessment, remain with the patient, call for assistance, perform an immediate ABCDE assessment and activate the appropriate clinical deterioration or emergency response pathway."*

Reassessment after intervention: *"Mr/Ms ___, earlier you rated your pain as _____/10. How would you rate your pain now?"* (Before: _____/10 → After: _____/10). *"I would assess whether the intervention has been effective and monitor for any adverse effects."*

STATION 1 CLOSING SCRIPT

"Thank you, Mr/Ms ___. I have completed your observations. Is there anything else you need at the moment? I will ensure that you are comfortable, your bed is in a safe position and your call bell and personal belongings are within reach. I would perform hand hygiene, document all observations and the pain assessment, communicate any relevant findings, and escalate abnormal findings according to the facility's clinical deterioration pathway."

OSCE Scripts — Station 2: Basic Life Support / DRSABCD

Scenario and D — Danger

You find a patient collapsed and unresponsive. Announce each step aloud for safety and marks. Confirm scene safety before contact.

STUDENT VERBALISES BEFORE APPROACHING:

"I am checking the environment for any danger to myself, the patient and others."

Identified hazards: **Spills / cords / obstacles cleared**

IF SAFE:

"The environment appears safe. I can approach the patient."

Station 2 Script — Response, Send, Airway, Breathing, CPR & AED

R — Response & S — Send for help:

"Hello, can you hear me? Open your eyes. My name is _____. Can you tell me your name? Can you squeeze my hands?"

If no response: *"The patient is unresponsive. Help! I have an unresponsive patient! Please activate the emergency response, call* **2222** *, and bring the resuscitation trolley and AED/defibrillator."*

A — Airway & B — Breathing:

"I am opening and assessing the patient's airway. I am checking whether the airway is clear and patent." (Airway: ☐ Patent / ☐ Obstructed).

"I am assessing whether the patient is breathing normally." (Breathing: ☐ Normal / ☐ Abnormal / ☐ Absent).

- **If breathing normally:** Maintain airway, recovery position, call for help, monitor breathing.
- **If NOT breathing normally (or agonal gasping):** *"The patient is unresponsive and is not breathing normally. I will commence CPR immediately."*

C — CPR & D — Defibrillation:

"I will commence chest compressions (30 compressions : 2 breaths, rate 100–120/min, depth 5–6 cm). I am allowing full chest recoil and minimising interruptions. I will attach the AED as soon as possible. Everyone stand clear. Do not touch the patient. Shock advised—visually checking nobody is touching the patient. Clear! Delivering shock. I will immediately recommence CPR beginning with chest compressions."

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 2: High-Quality Chest Compressions

Place interlocked hands on the lower half of the sternum. Compress 5–6 cm at 100–120/min with complete chest wall recoil.

OSCE Scripts — Station 3: Blood Transfusion

Student opening and Pre-transfusion checks

> *"Hello, my name is _____, and I am your student nurse/RN today. Could you please tell me your full name and date of birth?"*

Patient name: _____ | DOB: _____ | UR/MRN: _____

> *"Thank you, Mr/Ms _____. You have been prescribed a blood transfusion today. I need to complete several safety checks before we commence."*
>
> - *"Do you have any known allergies?" → Allergies:* _____
> - *"Have you ever received a blood transfusion before?" →* ☐ *Yes /* ☐ *No*
> - *"Have you ever experienced a reaction to blood or blood products?" →* ☐ *Yes /* ☐ *No*

Station 3 Script — Baseline Assessment & Reaction Management

Pre-transfusion & Baseline Assessment (Verbalise):
"I would verify the prescription and consent, confirm patient ID and compatibility documentation, inspect the blood product for expiry and integrity, and complete the required independent two-clinician check."

Blood product: _____ | Group: _____ | Unit #: _____ | Expiry: _____

Baseline Obs: Temp: _____ °C | HR: _____ bpm | RR: _____ /min | BP: _____ mmHg | SpO_2: _____ %

Patient Education & Suspected Reaction Script:

- *Education:* "Please tell me immediately if you develop fever, chills, shaking, itching, rash, difficulty breathing, chest or back pain, nausea, or feel unwell."
- **Patient states:** *"I'm feeling cold and I'm having trouble breathing."*
- **Student Action:** *"I am stopping the transfusion immediately. Mr/Ms ___, I am concerned that you may be experiencing a reaction to the blood transfusion. I am going to stay with you and assess you. I would immediately assess airway, breathing and circulation and repeat observations:"*

Temp: _____ °C | HR: _____ | RR: _____ | BP: _____ | SpO_2: _____ %

- *"I would maintain IV access according to protocol and would NOT flush the remaining blood product into the patient. I would urgently notify the senior nurse, medical team and blood bank, follow investigation procedures for blood/urine samples and the blood bag/giving set, and document."*

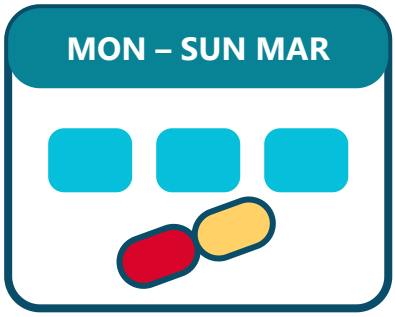
NEXTGEN NURSES VISUAL CLINICAL ANCHOR



Station 3: Immediate Clamping & Line Replacement

Never flush the blood remaining inside the giving set tubing into the patient! Disconnect at the cannula hub and attach a fresh 0.9% Sodium Chloride line.

NEXTGEN NURSES VISUAL CLINICAL ANCHOR



OSCE Scripts — Stations 4 and 5

Medication Administration & Intravenous Fluid Therapy Interactive Scripts

Station 4 — Medication Administration Script

Student opening: *"Hello, my name is _____, and I am your student nurse/RN today. I am here to administer your prescribed medication. Could you please tell me your full name and date of birth?"*

Name: _____ | DOB: _____ | MRN/UR: _____

Allergies: _____ | Previous reactions? ☐ Yes / ☐ No

Medication safety checks (Verbalise): *"I would check the medication order and confirm the correct patient, medication, dose, route, time and indication, plus allergies, expiry, contraindications, relevant observations/pathology, and independent checking."*

Medication: _____ | Dose: _____ | Route: _____ | Time: _____ | Indication: _____

PATIENT EDUCATION, REFUSAL & UNSAFE ORDER SCRIPTS:

- *Education: "You have been prescribed _____ mg of _____ via the _____ route to _____. Before I administer it, I need to check* **BP / BGL / Pain & RR** *. Are you happy to take the medication?"*
- *If Patient Refuses: "That's okay. Can you tell me what concerns you have? You have the right to refuse medication. I would not force the patient, and I would document and notify the prescriber."*
- *If Order is Unsafe/Unclear: "I would not administer this medication because I cannot safely proceed until the order has been clarified with the prescriber."*

Station 5 — IV Fluid Administration Script

Student opening & Prescription check:

"Hello, my name is _____, RN. Your prescriber has ordered IV fluids for you. Could you confirm your full name and DOB?" Name: _____ | DOB: _____

"Do you have any allergies? Are you experiencing any pain, burning or discomfort around your IV cannula?"

Fluid: _____ | Volume: _____ mL | Duration: _____ hrs | Additives: _____

IV site assessment & Rate calculation:

- Inspect for Pain, Redness, Swelling, Leakage, Warmth, Phlebitis, Infiltration. *"The IV site appears* **clean, dry, intact** *."*
- Formula: Total volume ÷ number of hours = mL/hr (e.g., 1000 mL ÷ 8 hrs = 125 mL/hr).
- *"The prescribed fluid is _____ mL over _____ hours. Therefore, I would set the infusion rate at _____ mL per hour."*

Possible fluid overload script (if dyspnoea develops):

"Mr/Ms ___, I am concerned about your new shortness of breath. I am going to reassess you immediately (RR, SpO_2, HR, BP, WOB, lung sounds, oedema, fluid balance). I would review/pause the fluid infusion immediately and escalate the patient's deterioration to the appropriate clinician."

OSCE Scripts — Station 6: ISBAR Clinical Handover

Student preparation — collect: Patient ID, Diagnosis, Relevant history, Current observations, Recent changes, Medications, Allergies, Assessment findings, and What you need from the clinician.

- [ ] **I — Identification:** *"Hello Dr/NP/RN _____. My name is _____, RN on _____ Ward. I'm calling about Mr/Ms _____, a _____-year-old patient in bed _____."*
- [ ] **S — Situation:** *"I am concerned because the patient has developed* **increasing shortness of breath and falling SpO** *."*
- [ ] **B — Background:** *"The patient was admitted with _____. Relevant medical history includes _____. Relevant medications include _____. The patient is allergic to _____."*
- [ ] **A — Assessment:** State current parameters then: *"My focused assessment identified _____."*
- [ ] **R — Recommendation:** *"I am requesting an urgent medical review. Could you please review the patient as soon as possible? While waiting, I will continue monitoring the patient and repeat observations. If the patient deteriorates further, I will activate the emergency response. Can I confirm the plan is _____?"*

Station 7 — Pain Assessment & Management (Scripted)

"Hello, my name is _____, and I am your student nurse/RN. I understand that you're experiencing some pain. I'd like to ask you some questions so I can understand your pain better. Is that okay?"

- **P — Provocation/Palliation:** *"What were you doing when the pain started? What makes it worse or better?"* Findings: _____
- **Q — Quality:** *"How would you describe the pain? (Sharp, dull, aching, burning, stabbing, crushing, throbbing?)"* Quality: _____
- **R — Region/Radiation:** *"Can you show me where the pain is? Does it spread anywhere else?"* Location: _____ | Radiation: _____
- **S — Severity:** *"On a scale from 0 to 10, how would you rate it?"* Pain: _____ /10
- **T — Timing:** *"When did it start? Sudden or gradual? Constant or intermittent?"* Timing: _____
- **Associated symptoms:** *"Are you experiencing any other symptoms?"* _____

MANAGEMENT & REASSESSMENT SCRIPT:

"I would obtain relevant observations, review the medication chart, and provide prescribed analgesia and non-pharmacological strategies. Earlier, you rated your pain as _____ /10. How would you rate it now? (After: _____ /10). I would document the intervention and response, and escalate if pain remains severe or worsens."

Station 8 — Focused Body-System Assessment (Respiratory)

> *"Hello, my name is _____, RN. I understand that you've been experiencing some difficulty breathing. I'd like to assess your breathing and ask you some questions. Is that okay?"*

Subjective assessment:

- *"When did the shortness of breath start?"* Onset: _____ (Sudden / Gradual | At rest / Moving)
- Chest pain? ☐ Yes ☐ No | Cough? ☐ Yes ☐ No | Sputum colour/amount: _____
- Home oxygen? ☐ Yes ☐ No | History (Asthma, COPD, Heart problems): _____

Objective assessment & Auscultation (Verbalise):

"I am assessing general appearance, skin colour, ability to speak and consciousness." Record RR, SpO_2, O_2 device, WOB, chest movement.

"I would auscultate the lung fields systematically and compare both sides." Findings: ☐ Clear ☐ Wheeze ☐ Crackles ☐ Reduced air entry.

Interpretation: *"I am concerned about _____ because _____. I would continue an ABCDE assessment and escalate according to deterioration criteria."*

OSCE Scripts — Station 9: Risk Assessment (Falls Example)

"Hello, my name is _____, RN. I would like to ask you some questions about your mobility and risk of falling so that we can keep you safe while you're here. Is that okay?"

• **Falls history:** Recent falls? ☐ Yes ☐ No | How many / when / injured?

_____

• **Mobility:** Walks independently? ☐ Yes ☐ No | Mobility aid: _____ | Needs assistance? ☐ Yes ☐ No

• **Dizziness / Vision / Toileting:** Dizzy when standing? ☐ Yes ☐ No | Vision problems / wears glasses? ☐ Yes ☐ No | Needs bathroom assistance / rushes to toilet? ☐ Yes ☐ No

• **Cognition:** Assess orientation, confusion, impulsivity, and ability to follow instructions.

FALLS-PREVENTION, PRESSURE-INJURY & DELIRIUM VERBALISATION:

• *Falls: "Based on my assessment, Mr/Ms _____ has an increased risk of falling. Please use your call bell before getting out of bed. I would ensure the bed is low, hazards are cleared, call bell and aids are within reach, and non-slip footwear is worn."*

• *Pressure Injury: "I would assess skin integrity, mobility, continence, nutrition and hydration using the facility tool, and implement repositioning and pressure-relieving equipment."*

• *Delirium: "I am concerned about this acute change in cognition. I would assess for infection, hypoxia, dehydration, pain, medication effects and metabolic disturbances and escalate."*

Station 10 — Wound Dressing & ANTT (Scripted – Part 1)

> *"Hello, my name is _____, RN today. I need to change the dressing on your _____ wound. Could you please tell me your full name and date of birth?"*

Name: _____ | DOB: _____

• *"Do you have any allergies, particularly to dressings, adhesives or cleansing products?"* Allergies: _____

• *"Are you experiencing any pain around the wound?"* Pain: _____/10 (Consider analgesia before commencing).

• *"I will explain the procedure as we go. Is it okay for me to proceed?"*

Preparation & Removing Old Dressing:

• *Verbalise:* "I would review the wound-management plan and prescribed dressing, gather all required equipment, provide privacy, position the patient appropriately and perform hand hygiene."

• *To patient:* "I'm going to remove your old dressing now. Please let me know if you experience any discomfort."

• *Verbalise:* "I am observing the old dressing for the amount, colour and type of exudate and any evidence of bleeding."

Old dressing findings: _____

Dispose appropriately and perform hand hygiene / change gloves as required.

Station 10 — Wound Assessment, ANTT & Closing (Part 2)

Wound Assessment (WOUND Mnemonic):

- **W (Wound bed — granulation, slough, necrotic):** _____
- **O (Abnormal odour):** _____
- **U (Surrounding skin — redness, warmth, swelling, maceration):** _____
- **N (Nature of exudate — amount, colour, consistency):** _____
- **D (Dimensions):** Length: _____ cm × Width: _____ cm × Depth: _____ cm

ANTT, DRESSING APPLICATION & ESCALATION:

"I will use Aseptic Non-Touch Technique and protect key parts and key sites throughout the procedure. I will avoid touching any key part or key site directly. I will clean the wound according to the prescribed plan and apply the prescribed dressing (_____)."

• If signs of infection present: "I am concerned about possible wound infection/deterioration. I would escalate these findings to the appropriate clinician."

STATION 10 CLOSING SCRIPT:

"Mr/Ms _____, your wound dressing has now been completed. How are you feeling? Are you experiencing any pain or discomfort? (Post-pain: _____/10). I will make sure you're comfortable and your call bell is within reach. I would dispose of equipment, perform hand hygiene, document the wound assessment, and escalate any abnormal findings."

Universal and Emergency Closing Scripts

UNIVERSAL OSCE CLOSING SCRIPT — ADAPT TO ALMOST EVERY STATION:

"Mr/Ms _____, I have completed your _____. Is there anything else you need at the moment? Are you comfortable? I will ensure that your bed is in a safe position and that your call bell and personal belongings are within reach."

STUDENT VERBALISES:

"I would perform hand hygiene. I would document the assessment/procedure and the patient's response. I would communicate relevant findings and escalate any abnormalities to the appropriate clinician."

Emergency script to memorise — If the patient suddenly deteriorates:

- *"I am concerned about this patient's clinical condition."*
- *"I would stop what I am doing, remain with the patient and call for assistance."*
- *"I would perform an immediate ABCDE assessment and repeat the patient's observations."*
- *"I would initiate appropriate interventions within my scope of practice and activate the appropriate clinical deterioration or emergency response pathway."*
- *"I would continuously reassess the patient while awaiting further assistance."*

Final NextGen Nurses OSCE Memory Formula

- **START:** HAND HYGIENE → INTRODUCE → IDENTIFY → ALLERGIES → EXPLAIN → CONSENT → PRIVACY.
- **DURING:** ASSESS → INTERPRET → INTERVENE → COMMUNICATE → SAFETY.
- **FINISH:** REASSESS → ESCALATE → DOCUMENT → HANDOVER.

Most important principle: Do not simply demonstrate a procedure. Show the examiner that you can recognise what the findings mean and know what you would do next to keep the patient safe.